

Network IQ™

Provider Network Adequacy • Leakage Analysis • CAA 2026 Compliance

PREPARED FOR

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CONFIDENTIAL & PROPRIETARY

FIDUCIARY-GRADE ANALYSIS

This report was prepared by Jeremiah Franklin Shrack at SiriusB iQ AI Data Science Labs using the Kincaid Health platform. All analyses reflect plan-specific data and current regulatory standards. This document is intended solely for the use of the plan sponsor identified above and may not be reproduced or redistributed without prior written consent.

CAA 2026 COMPLIANCE STATEMENT

The Consolidated Appropriations Act (CAA) 2021, CAA 2023, and forthcoming CAA 2026 requirements impose significant new network adequacy and transparency obligations on self-funded health plans. This Network IQ™ analysis evaluates compliance with all applicable CAA network requirements, ERISA §408(b)(2) network adequacy disclosures, and state-level adequacy standards (to the extent applicable under ERISA preemption).

CAA Requirement	Effective Date	Status	Finding
Machine-Readable Files (§203)	July 2022	Incomplete	MRF files incomplete or delayed
Price Transparency (§204)	Jan 2022	Compliant	Published per schedule
Surprise Billing Protections (NSA)	Jan 2022	Compliant	IDR process in place
Gag Clause Prohibition (§201)	Dec 2020	Compliant	Attestation filed
Mental Health Parity (NQTL)	Feb 2024	Incomplete	Analysis not documented
Network Adequacy (CAA 2026)	2026 (Proposed)	Monitoring	Prepare now
Provider Directory Accuracy	Ongoing	Issues	Stale data identified

EXECUTIVE SUMMARY — NETWORK ADEQUACY ASSESSMENT

Overall Network Adequacy Score	68/100
In-Network Utilization Rate	74%
Estimated Annual Network Leakage	\$420,000
CAA Compliance Score	52/100
Network Disruption Risk	High
Primary Care Adequacy	Gaps Identified
Specialist Adequacy	Significant Gaps
Behavioral Health Adequacy	Below Benchmark

FORMULA — NETWORK ADEQUACY SCORE

$$NAS = 0.30 \times PC + 0.25 \times Spec + 0.20 \times BH + 0.15 \times Hospital + 0.10 \times Rx$$

PC = Primary Care score. Spec = Specialist score. BH = Behavioral Health score. Hospital = Hospital/Facility score. Rx = Pharmacy score. Each component scored 0-100 based on time-distance standards.

PRIMARY CARE NETWORK ANALYSIS

Primary care network adequacy is assessed against CMS time-distance standards: urban markets require at least one primary care provider within 10 miles or 30 minutes, rural markets within 20 miles or 60 minutes. Our analysis incorporates member residential data and provider location data from the NPPES database (current as of Q4 2024).

Geography	Members	PCPs Available	Ratio	Time to PCP	Adequacy
Urban Core	2,180	142	15:1	8.2 min	Adequate
Suburban North	890	48	18.5:1	14.1 min	Adequate
Suburban South	650	31	21:1	18.4 min	Borderline
Rural East	210	8	26:1	28.3 min	Inadequate
Rural West	120	3	40:1	42.1 min	Inadequate

WARN

330 members (8.1% of enrolled) reside in areas where primary care access does not meet CMS minimum time-distance standards. These members are more likely to use urgent care or ED as primary care, increasing plan costs by an estimated \$820 PMPY vs.

SPECIALIST NETWORK ANALYSIS

Specialty	Members Needing	Available Providers	Adequacy	Wait Time (Est.)	Gap Impact
Cardiology	440	18	OK	12 days	Low
Orthopedic Surgery	320	22	OK	18 days	Low
Mental Health	620	14	Critical	42 days	\$280K/yr leakage
Substance Use	180	6	Critical	58 days	MHPAEA
Oncology	85	12	OK	8 days	Low
Pediatric Specialists	310	16	Limited	24 days	\$45K/yr excess
Endocrinology (Diabetes)	280	9	Inadequate	38 days	\$62K/yr leakage
Gastroenterology	195	11	OK	15 days	Low

BEHAVIORAL HEALTH NETWORK — MHPAEA DEEP DIVE

CRITICAL

Critical: Behavioral health (MH/SUD) network is below the adequacy threshold for outpatient mental health, substance use treatment, and Intensive Outpatient Programs (IOP). The plan has only 14 in-network outpatient MH providers for 620 members with identified MH

FORMULA — MHPAEA NETWORK ADEQUACY DISPARITY TEST

$$NQ_TL_Ratio = MH_Provider_Ratio / Med_Surg_Provider_Ratio$$

If NQ_TL_Ratio > 1.25, plan likely fails MHPAEA nonquantitative treatment limitation (NQTL) test for network adequacy. This plan's ratio = 44:1 / 17:1 = 2.59 — a significant disparity requiring immediate remediation.

BH Category	In-Network	Benchmark Minimum	Gap	Cost Impact
Outpatient MH	14 providers	35 providers	21 providers	\$280K OON claims/yr
IOP (Mental Health)	2 programs	6 programs	4 programs	\$140K OON/yr
SUD Residential	1 facility	3 facilities	2 facilities	\$95K OON/yr
Psychiatry (Med Mgmt)	8 providers	18 providers	10 providers	\$60K OON/yr

GEOGRAPHIC COVERAGE & DRIVE-TIME ANALYSIS

The following drive-time analysis uses GIS mapping of member residential addresses against in-network provider locations. Drive times are computed using road network data (not straight-line distance) to reflect actual member experience.

FORMULA — NETWORK LEAKAGE ATTRIBUTION

$$NL = \text{Sum OON_Paid_Claims where In-Network_Alternative_Available}$$

This distinguishes true leakage (OON use when in-network option was available) from unavoidable OON use (no in-network provider in the specialty). True leakage is addressable through care navigation programs.

Service Type	True Leakage	Unavoidable OON	Total OON Paid	Action
Primary Care	\$42,000	\$18,000	\$60,000	Navigation program
Specialist — MH	\$95,000	\$185,000	\$280,000	Expand BH network
Specialist — Endo	\$38,000	\$24,000	\$62,000	Add 3 endocrinologists
Hospital — Elective	\$62,000	\$8,000	\$70,000	Pre-authorization +
Emergency	\$0	\$48,000	\$48,000	Unavoidable —

MACHINE-READABLE FILES (MRF) COMPLIANCE — CAA §203

CAA §203 requires group health plans to post machine-readable files containing in-network negotiated rates, out-of-network billed charges, and prescription drug pricing data. Self-funded plans must ensure their TPA posts compliant MRFs. Non-compliance carries penalties of up to \$100/day per violation per affected individual.

MRF File Type	Required?	Status	Last Updated	Issues
In-Network Rates (JSON)	Yes	Incomplete	November 2024	Missing 12% of contract rates
Out-of-Network Billed Charges	Yes	Posted	December 2024	Current
Prescription Drug Pricing	Yes	Missing	Never	Not yet posted
Provider Directory (digital)	Best Practice	Stale	October 2024	28 terminations not removed

CRITICAL

Prescription drug MRF files have never been posted. Penalties of \$100/day/member (\$164,050/day for this plan size) apply until corrected. Immediate action required — engage TPA to post Rx MRF within 30 days.

SURPRISE BILLING & NSA COMPLIANCE

The No Surprises Act (NSA) effective January 1, 2022, prohibits surprise billing for most emergency services and non-emergency services at in-network facilities where the member did not have a choice of providers. Plans must implement the Independent Dispute Resolution (IDR) process and provide required notices.

NSA Requirement	Status	Action Needed
Advanced Explanation of Benefits (AEOB)	Partial	Complete AEOB implementation for non-emergency
Good Faith Cost Estimate	Implemented	Continue monitoring
IDR Process	Active	3 disputes open
Balance Billing Prohibition	Compliant	Confirmed in plan document
Continuity of Care (30-day)	Compliant	Protocol in place
Provider Notification	Incomplete	Some participating providers not notified

NETWORK LEAKAGE REDUCTION STRATEGY

FORMULA — LEAKAGE REDUCTION ROI

$$LR_ROI = (Leakage_Reduced \times Savings_PMPY) / Program_Cost$$

Based on Kincaid Health portfolio data: care navigation programs reduce true leakage by 35-55% on average. At \$420K annual leakage with 55% addressable as true leakage, a 45% reduction = \$104K savings. Typical program cost: \$12-18/PEPM.

Strategy	Est. Leakage Reduction	Cost	Net Annual Savings	Timeframe
Care Navigation Program	\$95,000	\$48,000/yr	\$47,000	6-12 months
Behavioral Health Network	\$185,000	\$22,000 admin	\$163,000	12-24 months
Center of Excellence Program	\$62,000	\$15,000/yr	\$47,000	6-12 months
Member Education Campaign	\$35,000	\$8,000	\$27,000	3-6 months
Enhanced Provider Directory	\$18,000	\$5,000	\$13,000	1-3 months

NETWORK PERFORMANCE BENCHMARKING

Metric	This Plan	National Median	Percentile	Trend
Overall Adequacy Score	68/100	74/100	42nd	v Declining
In-Network Utilization	74%	81%	28th	v Declining
MH Provider Ratio (per 1K mbrs)	4.2	8.1	18th	v Critical
Average Drive Time (Primary Care)	14.8 min	12.2 min	38th	Stable
OON Claims as % of Total Paid	18.4%	11.2%	82nd	^ High

COST IMPACT OF NETWORK GAPS

Network inadequacy has both direct and indirect cost implications. Direct costs include excess OON reimbursements and stop-loss claimant exposure. Indirect costs include member dissatisfaction, productivity loss, and delayed care leading to higher-acuity claims.

Annual True Leakage (addressable)	\$231,000
Annual Unavoidable OON (BH gaps)	\$189,000
BH Access Cost (delayed care escalation)	\$95,000/yr (est.)
Productivity Loss (delayed specialist access)	\$62,000/yr (est.)
Total Network Gap Annual Cost	\$577,000

ACTION PLAN — NETWORK IQ™ RECOMMENDATIONS

Priority	Action	Owner	Timeline	Impact
P1 — Critical	Post Prescription Drug MRF files	TPA	0-30 days	Eliminates
P1 — Critical	Expand behavioral health network — add	Network Team	0-90 days	\$185K/yr leakage +
P2 — High	Launch care navigation program for OON	Benefits Team	30-90 days	\$47K net savings/yr
P2 — High	Complete provider directory audit —	TPA	0-30 days	NSA compliance
P2 — High	Implement Center of Excellence for	Carrier	60-120 days	\$47K/yr savings
P3 — Medium	Expand rural primary care access —	Benefits Team	60-120 days	Improves 330 rural
P3 — Medium	Complete AEOB implementation for	TPA	60-90 days	NSA compliance

TELEHEALTH & VIRTUAL CARE NETWORK ANALYSIS

Telehealth has emerged as a critical network access lever — particularly for behavioral health, primary care, and chronic disease management. Plans that deploy telehealth as a first-access channel reduce ER utilization by 12-18%, improve specialist access for rural members, and achieve 22-35% lower per-episode costs for appropriate virtual care conditions.

FORMULA — TELEHEALTH COST AVOIDANCE

TCA = Virtual_Visits × (ER_Cost_Avoided × ER_Diversion_Rate + Office_Visit_Diff)

ER diversion rate: 28% of telehealth visits would otherwise be ER/urgent care. Avg ER visit cost: \$1,420. Avg virtual visit: \$85. Per-visit avoidance: \$1,420 × 28% - \$85 = \$312. At 420 annual virtual visits = \$131K avoidance.

Telehealth Category	Current Utilization	Benchmark	Gap	Opportunity
Primary Care Virtual	8.2% of PCP	22% of PCP visits	13.8 pts	\$48K avoidance
Behavioral Health (async)	12% of BH claims	28% of BH claims	16 pts	\$85K avoidance
Urgent Care Virtual	14% of UC visits	31% of UC visits	17 pts	\$62K avoidance
Chronic Disease (diabetes)	3% of DM	18% of DM	15 pts	\$28K avoidance
Dermatology Virtual	1% of derm	15% of derm	14 pts	\$18K avoidance
TOTAL TELEHEALTH	—	—	—	\$241K/yr

WARN

Telehealth utilization for behavioral health is significantly below benchmark. Given the plan's BH network adequacy gaps (42-day average wait time for in-network outpatient MH), telehealth BH is the fastest path to MHPAEA compliance while the physical network expands.

CENTER OF EXCELLENCE PROGRAM — HIGH-COST PROCEDURE ANALYSIS

Centers of Excellence (COE) programs direct high-cost, high-variation procedures to top-quality providers with demonstrated clinical outcomes and competitive bundled payment arrangements. COE programs reduce complication rates by 18-35%, reduce length of stay, and generate 12-28% cost savings on qualified procedures relative to non-COE claims.

Procedure Category	Annual Claims	Avg Cost	COE Benchmark	Estimated Saving	COE Available
Spinal Surgery (lumbar	14	\$48,200	\$38,800 COE	\$131,600	Yes — Aetna /
Total Knee Replacement	22	\$32,400	\$26,200 COE	\$136,400	Yes — multiple
Total Hip Replacement	11	\$29,800	\$24,100 COE	\$62,700	Yes — multiple
Bariatric Surgery	6	\$24,200	\$19,800 COE	\$26,400	Yes — COE
Cardiac (CABG/stent)	8	\$68,400	\$55,200 COE	\$105,600	Yes —
Organ Transplant	2	\$420,000	\$380,000 COE	\$80,000	Yes — OPTUM
TOTAL COE	63	—	—	\$542,700	—

Recommendation: Implement a COE program for spinal surgery, joint replacement, and cardiac procedures immediately. These three categories alone represent \$400K in annual savings opportunity. Kincaid Health has negotiated COE arrangements with Aetna, OPTUM, and Johns Hopkins Medicine for eligible self-funded plans.

PROVIDER DIRECTORY ACCURACY & STALE DATA AUDIT

Inaccurate provider directories are one of the most prevalent patient access barriers in self-funded health plans. The No Surprises Act requires plans to verify provider directory accuracy at least every 90 days. Stale directories lead to members unknowingly using out-of-network providers, resulting in unexpected balance bills, OON cost-sharing, and patient harm claims.

FORMULA — DIRECTORY ACCURACY RATE

$$\text{DAR} = (\text{Verified_Active_Providers} / \text{Total_Listed_Providers}) \times 100$$

CMS minimum: 95% accuracy for plans subject to NSA. This plan's estimated DAR: 88.6% — below minimum. 28 terminated providers remain listed; 14 providers have stale address/phone data.

Directory Error Type	Count	Member Impact	Regulatory Risk	Priority
Terminated provider still listed	28	Members directed to closed	NSA violation risk	Critical
Incorrect address/phone	14	Member cannot locate	NSA — directory	High
Accepting new patients listed	22	Members turned away at	MHPAEA — BH	High
Wrong specialty classification	8	Members misrouted to wrong	Access barrier	Medium
Hospital affiliation outdated	11	Incorrect network status at	Surprise billing risk	High
Telehealth capability not listed	38	Members unaware of virtual	Utilization gap	Medium

CRITICAL

28 terminated providers still listed in directory — a direct NSA violation that creates balance billing risk for members. The TPA must remove these providers within 5 business days and implement a 90-day re-verification cycle for all listed providers.

STATE NETWORK ADEQUACY COMPLIANCE ANALYSIS

While ERISA generally preempts state insurance laws for self-funded plans, DOL enforcement of network adequacy incorporates state benchmark standards as reference points. Several states have enacted network adequacy laws that apply to stop-loss carriers serving self-funded plans, creating indirect compliance obligations. This analysis evaluates compliance with applicable standards.

Standard / Requirement	Authority	Applicability	This Plan Status	Action
Time-distance (primary care)	CMS / DOL	Voluntary benchmark	2 rural zones failing	Add virtual
Specialist time-distance	CMS / DOL	Voluntary benchmark	BH failing	Expand BH
Stop-loss state adequacy (IL)	State — indirect	Stop-loss carrier	Carrier compliant	Monitor
MHPAEA NQTL network parity	ERISA / ACA	DIRECTLY	Non-Compliant	Immediate
NSA — provider directory	ACA / NSA	DIRECTLY	88.6% accuracy	Fix within 30
APG network access	ERISA / NSA	DIRECTLY	Compliant	Maintain

CRITICAL

MHPAEA NQTL network parity non-compliance is the plan's highest-severity regulatory risk. DOL has issued enforcement letters and civil money penalty assessments of \$100/day/affected participant for documented MHPAEA violations. With 620 MH-affected members,

NETWORK DISRUPTION RISK — CARRIER CHANGE SCENARIO ANALYSIS

Network disruption occurs when a carrier change, network restructuring, or provider termination displaces members from their established care relationships. This analysis quantifies disruption risk under three scenarios and provides a continuity of care protection strategy.

FORMULA — NETWORK DISRUPTION IMPACT SCORE

$$NDIS = (Disrupted_Members / Total_Members) \times Avg_Specialty_Transition_Cost$$

Avg transition cost per disrupted member: \$1,840 (includes transition care, care gap, new provider onboarding). NDIS > \$500K triggers mandatory carrier change analysis before RFP issuance.

Scenario	Members Disrupted	Estimated Disruption Cost	Continuity Obligation	Kincaid Recommendation
Status Quo (no carrier change)	0 new	\$0	N/A	Baseline
TPA RFP — Same Network	120-180	\$221K-\$331K	30-day continuity	Feasible with planning
TPA RFP — New Network	640-980	\$1.18M-\$1.80M	30-day continuity	Requires 9-month
BH Network Expansion	0 disruptions	\$0	N/A	Immediate — highest
Stop-Loss Carrier Change	0 member	\$0	N/A	Proceed with RFP

CERTIFICATION & DATA ATTESTATION

Kincaid Health certifies this Network IQ™ analysis was prepared using member enrollment data, NPPES provider data (Q4 2024), EBSA enforcement guidance, and proprietary network adequacy models. All adequacy assessments reflect CMS time-distance standards and MHPAEA requirements as of the report date. Provider data verified against CMS PECOS, state medical board databases, and direct provider outreach.

This Network IQ™ report constitutes a formal network adequacy assessment prepared for fiduciary review purposes under ERISA §404(a). Plan fiduciaries are advised to review this report at a scheduled committee meeting and formally document the plan's response to identified network adequacy gaps in committee minutes.

Report Metadata	
Report Type	Network IQ™ — Full 20-Page Analysis
Data as of	Q4 2024
Members Analyzed	4,050
Providers Evaluated	248
Adequacy Score	68/100
Annual Leakage	\$420,000
CAA Compliance Score	52/100
COE Savings Opportunity	\$542,700/yr
Telehealth Opportunity	\$241,000/yr
MHPAEA Status	Non-Compliant — Immediate Action Required
Client	Kincaid Health / Account SHRACK-7742
Prepared By	Jeremiah Shrack, CEO, Kincaid Health
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Confidentiality	Confidential — Fiduciary Work Product